SCOTTISH DENTAL MARKETPLACE

Product white paper and technical specification for website developers

Version: 1.0 | Date: 20 October 2026 | Market: Scotland first, UK-ready

Reference concept: https://www.openwide.clinic

Confidential: For development scoping, technical evaluation and quotation.

Core policy: Practice information will be researched and stored privately for direct outreach. No practice will be publicly listed without authorisation, verification and approval.

1. Executive summary

Build a Scotland-first online marketplace connecting people seeking dental care with participating, verified practices. Patients will be able to discover practices by location, treatment, NHS/private offering, verified prices and availability. Practices will receive a self-service profile and enquiries dashboard. The platform operator will receive an internal acquisition CRM, verification workflow, commercial controls and analytics.

This is a marketplace software product, not merely a practice directory or brochure website. It must be mobile-first, accessible, searchable, secure and designed to expand throughout the UK without rebuilding the underlying data model.

2. Non-negotiable principles

- No automatic public population: imported or researched Scottish practices remain private prospects, not public listings.
- Practice consent and verification: a public profile is created only after authorised practice enrolment and administrative approval.
- Truthful information: treatment prices, services, NHS acceptance, clinician credentials and availability must be confirmed and dated.
- Clear booking status: enquiry requests must never be labelled confirmed appointments.
- Separation of systems: internal prospect CRM and live public marketplace records must have distinct permissions and publishing controls.
- Compliance by design: privacy, marketing, cyber security, clinical advertising and accessibility must be considered from the start.

3. Users and permissions

Role	Key capabilities
Patient / visitor	Search, filter, compare, enquire; optional account.
Practice owner / authorised admin	Manage location profiles, prices, staff, enquiries and billing.
Practice team member	Respond to enquiries and manage permitted practice content.
Business development	Research and verify private prospects; log outreach and follow-ups.
Platform administrator	Approve/suspend listings, govern data, manage subscriptions and moderate content.
Super administrator	Access controls, audit, system settings and multi-tenant oversight.

4. Public marketplace - functionality

4.1 Homepage and navigation

Homepage: postcode/town search, treatment selector, NHS/private filter, featured verified practices, prominent patient and practice journeys, educational content and local search pages. Main navigation: Find a Dentist; Treatments; Emergency Dentistry; For Practices; Help; Sign In.

4.2 Search and results

- Location-based search with postcode geocoding, adjustable distance and map/list view.

- Filters: treatments, NHS/private, accepting new NHS patients (practice confirmed), prices, opening hours, verified clinician, accessibility, languages, sedation and appointment mode.
- Results show practice name, location, verified services, price labels, enquiry/booking status and last-confirmed availability where relevant.
- Ranking primarily by relevance and distance; sponsored placements transparently labelled.
- Zero-result states suggest broader radius or related treatments rather than invented listings.

4.3 Practice profile

SEO-friendly slug; photographs; contact details; verified address; services; NHS/private declaration; published treatment prices and conditions; clinician biographies; GDC numbers where applicable; opening hours; facilities; directions; patient enquiry action; optional reviews once moderation and authenticity controls exist. All publicly claimed services must be practice-approved.

4.4 Treatment directory

Create structured, indexable treatment pages for general and emergency dentistry, hygiene, restorative, cosmetic, orthodontic, implants, oral surgery and sedation. Patient-facing treatment descriptions must explain limitations and avoid claims of guaranteed suitability or outcomes.

5. Scottish prospect acquisition CRM

This is a private back-office system to collect and work through Scottish dental practices individually. It is not the public directory. Research should use appropriate official public records, practice websites and other lawfully reusable sources; the team should assess source terms, licensing, accuracy and marketing eligibility.

5.1 Prospect record fields

Data	Notes
Identity	Unique ID; practice name; address; postcode; local authority; NHS board; website.
Contact	General business email/phone; named contact only where lawfully gathered; contact preferences.
Research governance	Source URL/type; date captured; researcher; last verification date; provenance.
Segmentation	NHS/private status as unverified until checked; services; region; organisation group.
Outreach	Assigned account manager; status; attempts; notes; next-contact date; response.
Compliance	Opt-out/suppression flags, marketing basis, deletion/retention logic.
Conversion	Invitation token; registration; authority verification; approval; launch date.

5.2 CRM workflow

Research identified > record deduplicated > details verified > awaiting contact > initial outreach > interested/declined/follow-up > invitation sent > account registration > ownership/authority verified > profile completed > administrative review > published. Parallel states: no response, do not contact, suspended, verification failed and archived.

5.3 CRM features

- Manual entry and permission-controlled CSV import/export, with duplicate matching on business identity, address, website and telephone.

- Regional filters (council/NHS board/postcode), saved queues, bulk assignment, task reminders, notes, contact history and funnel reporting.
- Single-use or expiring invitation links, approved templates, audit trail, suppression list and controlled follow-up scheduling.
- Never automatically email a scraped list; communications must be authorised under applicable UK GDPR/PECR requirements.

6. Practice onboarding and verification

An invited authorised representative creates an account, verifies email, asserts authority, confirms practice details and accepts current platform terms. An administrator reviews evidence and approves publication. Email-domain control by itself is not sufficient where authority is uncertain. Each location has independent approval and publication state.

Practice owners can edit content; material changes may return fields for review. Provide change history, ownership dispute resolution, publishing preview, suspension, deactivation and termination workflows.

7. Practice dashboard

- Profile editor with images, services, clinician biographies, credentials, opening hours, access details and prices.
- Enquiry inbox showing received, acknowledged, contacted, converted, closed and declined states.
- Appointment configuration: enquiry only in MVP, manual availability later, integrations in subsequent releases.
- Reporting on profile views, search appearances, enquiry volume, response time and attributable conversion where measurable.
- Team roles, notifications, billing, invoices, account security and support requests.
- Multi-location parent-company accounts as an extensible, later-stage feature.

8. Patient enquiry and booking experience

MVP: patient submits a minimal enquiry containing contact details, preferred location/treatment and suitable contact preferences. The practice receives a secure notification and is responsible for contacting the patient. The interface must say clearly: “This is an appointment request, not a confirmed booking.” Provide anti-spam controls, acknowledgement, consent/privacy information, retention rules and enquiry audit trail.

Phase 2: practice-managed appointment availability, pending/reserved/confirmed/cancelled states, expiration/locking, notifications and rescheduling. Phase 3: optional real-time practice management system integrations, subject to vendor API availability, commercial permissions and robust synchronisation logic.

9. Commercial model

Tier	Illustrative offer
Free	Verified basic listing, practice information and controlled enquiry access.
Professional - e.g. £39/month	Enhanced profile, more content and richer enquiry analytics.
Premium - e.g. £79/month	Optional marketing modules and advanced analytics.
Multi-site - bespoke	Group governance, consolidated reporting and enterprise terms.

Prices above are hypotheses for testing, not approved tariffs. The administration area must manage trials, plan changes, promotional periods, VAT settings, invoices, failed payments, cancellation and entitlements. Paid placements must be marked as advertising. Referral or booking-based charging needs legal and regulatory review before activation.

10. Technical architecture and data model

Suggested stack (developer may propose equivalent): Next.js/React frontend; typed backend using Node.js/NestJS or comparable framework; PostgreSQL relational database with geospatial extension; hosted object storage for approved media; established authentication provider; search indexing as volume requires; managed queue for email jobs and webhooks; reputable transactional email and payment vendors.

Major entities: ProspectPractice, ProspectSource, OutreachActivity, Suppression, Invitation, Organisation, LivePractice, PracticeLocation, PracticeUser, Clinician, Treatment, PracticeTreatment, Price, ServiceAvailability, Enquiry, Booking, Subscription, Payment, Review, AuditEvent and ContentPage. Each public LivePractice must link to an approved account, not simply an imported prospect.

Use clearly defined API contracts, background job retry rules, idempotent payment/webhook handling, secure file uploads, access-control enforcement and disaster recovery procedures. Environments must be separated into development, staging and production.

11. Data protection, security and governance

- Complete UK GDPR privacy-by-design assessment and determine controller/processor roles. Publish appropriate privacy and cookie notices; implement consent management where required.
- Enforce role-based access controls with tenant isolation, MFA for privileged roles, encryption in transit/at rest, backups, vulnerability patching and rate limiting.
- Minimise patient enquiry data; do not collect clinical histories or special-category medical information in MVP unless necessary and appropriately governed.
- Maintain audit logs, documented retention/deletion schedules, subject rights workflows, breach management and processor agreements.
- Check GDC-related professional titles and any stated specialist status before publication; avoid false assurances or clinical endorsements.
- Meet WCAG 2.2 AA as a target, with keyboard operation, semantic labels, screen-reader support and accessible forms.
- Provide transparent NHS acceptance declarations that are last confirmed by the practice; do not treat NHS participation as evidence of current capacity.

12. SEO, content and performance

Implement server-rendered, indexable pages for enrolled practices and relevant geographic/treatment combinations; canonical URLs, structured data where appropriate, XML sitemaps, robots handling and redirects. No indexed prospect pages. Use performant responsive images, Core Web Vitals monitoring, caching, clean internal linking and editable metadata. Editorial treatment content should be reviewed for accuracy and periodically updated.

13. User experience and design

A premium but reassuring clinical design: high-contrast typography, generous spacing, calm neutral palette, clear mobile search, recognisable trust indicators and consistent calls to action. Avoid clutter and overly aggressive promotional claims. Provide design system, responsive Figma screens, components, error/empty states and accessibility specifications. Do not clone OpenWide visual assets or copyrighted design materials.

14. Analytics and KPIs

Area	Measures
Acquisition	Prospects researched/verified; outreach attempts; response rate; invitation acceptance.
Onboarding	Invitations to registration; verification completion; profiles live; time to go live.
Marketplace	Organic visits; search-to-profile CTR; enquiry conversion; zero-result searches.
Practice value	Enquiries per location; response time; confirmed bookings where trackable.
Business	Free-to-paid conversion; MRR; churn; ARPU; acquisition cost; support load.

Use privacy-conscious analytics. Track funnel events and distinguish proven conversions from self-reported outcomes. Produce exportable owner-level dashboards.

15. Delivery phases and priorities

Phase	Deliverables
0 - Discovery	Stakeholder interviews; detailed requirements; wireframes; legal/data review; source licensing; technical architecture; acceptance criteria.
1 - MVP	Private Scottish CRM; invite workflow; verification; public search; approved practice profiles; enquiry forms; practice dashboard; admin console; basic analytics.
2 - Commercial growth	Billing/subscriptions; enriched filters; optional reviews; richer reporting; manual appointment availability; multi-site features.
3 - Integrations	Authorised booking integrations; advanced scheduling; automated messaging; partner APIs; UK geographical expansion.

16. MVP acceptance criteria

- Unapproved imported prospects cannot appear on any public page, API, sitemap or search index.
- A practice can receive an invitation, register, verify authority, complete a profile and be approved without developer intervention.
- A patient can search by location and treatment and reach a verified practice page on mobile.
- An enquiry is delivered securely, recorded once, acknowledged and clearly shown as unconfirmed.
- Practice users cannot view or modify another organisation’s private data.
- Administrator can suppress a prospect, audit changes, suspend a listing and export approved operational reports.
- Core pages pass agreed accessibility, security, browser compatibility and load testing thresholds.
- The website is deployed with monitoring, backups, support documentation and recoverability checks.

17. Developer deliverables

- Discovery report; scope statement; requirements traceability matrix and prioritised user stories.
- Figma wireframes and final responsive designs; reusable design system.
- Version-controlled source code, deployment pipeline, infrastructure configuration and API specifications.
- Database ERD, import schemas, permissions matrix, moderation workflow and migration plan.
- Automated tests, manual QA records, accessibility assessment and security testing.
- Administrative and practice-user guidance; handover, code ownership terms, hosting responsibilities and support SLA.
- Itemised fixed-price/milestone quotation showing exclusions, third-party subscriptions and ongoing maintenance costs.

18. Request for proposal - vendor questions

- Which framework and hosting design do you recommend, and why?
- How will private acquisition records be kept completely separate from live listings?
- How will practice authority and GDC-related claims be verified?
- What is included in the MVP, and which items are excluded or third-party dependent?
- What third-party costs, licensing restrictions and ongoing support fees apply?
- How will consent, contact suppression, audit, privacy and retention be implemented?
- What are the milestone acceptance conditions, intellectual property arrangements and handover provisions?
- Can you demonstrate prior delivery of multi-tenant marketplaces, CRM systems and high-volume location search?

19. Risks and dependencies

Principal risks: availability and reuse rights of Scottish practice data; insufficient practice participation; practice verification fraud; unmaintained prices/NHS acceptance status; referral-fee regulatory concerns; third-party booking API restrictions; data protection compliance; and mismatch between advertised booking and actual appointment capacity. Address these through controlled sourcing, explicit onboarding, change reminders, clear labelling and staged integrations.

20. Recommended launch strategy

Start with a well-maintained private prospect CRM, recruit a focused cohort of willing practices in selected Scottish regions, and validate whether patients successfully find and contact suitable providers. Launch the consumer search experience only with verified enrolled practices. Improve results density, conversion and practice value before expanding nationwide. Maintain “Scotland-first, UK-ready” architectural flexibility from day one.

21. Final commissioning instruction

The commissioned developer must deliver two separate but integrated products: (1) a private Scottish dental-practice research and outreach CRM and (2) a public marketplace populated exclusively by enrolled, verified and approved practices. The initial launch prioritises discovery, profiles and enquiries; real-time bookings and integrations are later-stage upgrades. All commercial prices, claims and integration assumptions require validation during discovery.